Policy Recommendations For Safe Menstrual Practices in the New York State Education System

The Red Poppy Initiative | July 2026

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1. Executive Summary

The 2026 Legislative Season concluded with many bills passed and many more proposed. Following the closure of the Season, opportunities arise for legislators to speak with their constituents and hear personal stories in order to realize communities' wishes and solve statewide issues the following year. Though much of the population experiences menstruation for a significant portion of their lives, conversation about menstrual health and ensuring safe menstrual practices has often been dismissed due to social stigma that affects how menstruation is represented and introduced in the legislature. The topic of menstruation has been characterized as inappropriate and taboo, but it is imperative menstruation be discussed to improve the quality of life for any who experience it.

Menstruation, especially in the context of educational institutions, is a topic of utmost importance to The Red Poppy Initiative. As an organization composed of young, student women, we have lived through the existing social stigma regarding menstruation. Our concern arises from a lack of conversation regarding menstruation and women's issues, and we aspire to break the silence regarding women's health through legislative change and social discourse.

As part of our continued efforts to foster detailed and representative discussion on issues regarding menstruation, The Red Poppy Initiative brings in the unique perspectives of youth who have experienced school systems' attitudes and actions to address concerns regarding menstruation. This brief focuses on mandated curricula and destigmatization efforts in public schools, exploring existing legislation, currently proposed bills, and future policy recommendations. It begins by discussing legal requirements for public school health curricula then moves into more detail about fostering a beneficial classroom environment. We hope, through this policy brief, legislators may hear the voice of youth experiencing firsthand the impacts of how schools address menstruation, and consider potential methods that can help those who menstruate feel comfortable and safe in an educational environment.

This brief uses the term "women" to include individuals who menstruate and have a cervix. Additionally, this brief may cite experiences that students have had in public schools in the state of New York. For the sake of privacy, no schools nor students will be explicitly named.

2. Mental and Physical Symptoms of Menstruation

2.1 Mental Symptoms

When acknowledging the different effects that menstruation can bring to women, mental impacts are oftentimes overlooked. It is imperative to recognize the potential mental burdens or changes people who menstruate may experience to create a healthy and supportive school environment for all students.

2.1.1 Mental Implications of Menstruation

Because menstruation is not just an isolated period of time, but instead a typically monthly cycle of hormonal and physical changes, those who menstruate will experience a multitude of complicated and varying emotions throughout their cycle that will not be covered in this brief. This brief will include a generalized explanation of the mental implications of menstruation that will not highlight all the effects students may feel.

Prominently, the brief period of time before menstruation may start oftentimes invokes a series of mental effects, otherwise more commonly known as premenstrual symptoms or PMS.[1] The severity of PMS varies between individuals, but for individuals who are already suffering from mental health conditions like depression or anxiety, their conditions can often worsen during this time period.[2] Additionally, as a result of hormonal changes and shifts, those who menstruate may experience more severe mood swings or irritability throughout their menstrual cycle.[3]

2.1.2 Mental Effects in Education

These implications often affect a student's ability to engage in the classroom. During PMS, students may have trouble concentrating, feel anxiety or increased tension, stress regarding schoolwork, or be more prone to fatigue.[4] Furthermore, studies have been able to find a distinct correlation between the menstrual cycle and classroom participation, such as a more negative academic performance in general.[5] While not all women may experience these symptoms, and their severity varies between people, menstruation and the menstrual cycle could affect a student's participation in class.

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[1] Molly Smith, "How the Menstrual Cycle Impacts Mental Health," Loma Linda University Health, May 13, 2024.
[2] Ibid.
[3] "Premenstrual Syndrome (PMS) — Symptoms & Causes," Mayo Clinic, Feb. 25, 2022.
[4] Ibid.
[5] Sulagna Maity et al., "Academic and Social Impact of Menstrual Disturbances in Female Medical Students: A Systematic Review and Meta-Analysis," Frontiers in Medicine 9 (2022).

Educators may criticize students for being tired in their class during the week, or be negligent in providing deadlines to students who may be in extreme pain from menstrual symptoms. The cause for this is not just a lack of understanding and care from the educator themself, but also an overbearing social stigma that prevents women from speaking up and advocating for their own mental health. Therefore, one action cannot fully solve this issue, and many steps will need to be taken.

Female students in particular oftentimes feel silenced or unacknowledged when they try to seek health only to be asked if they are “on their period”; this phenomenon is particularly true both in the educational context and throughout every aspect of life. Using menstruation as an explanation for all behaviors perpetuates sexist stereotypes that cause women to feel uncomfortable speaking up for themselves, completing a self-fulfilling prophecy.[6] Thus, it is crucial for educators to not make baseless assumptions about their students or friends, but instead be open to listening and empathizing with individuals. The implementation of these recommendations, and simply being aware of the social biases we may carry, will not only create a healthier educational environment for women to feel comfortable in but also be able to grow as students.

2.2 Physical Symptoms

Menstruation and the menstrual cycle can drastically vary between women. Again, the content following this notice will be a generalization of the many physical symptoms menstruation may affect, and not all women experience menstruation the same.

2.2.1 Physical Implications of Menstruation

Those who menstruate experience a wide variety of physical symptoms. Most commonly, during menstruation, individuals may experience severe cramping in their abdomen.[7] These are often known as period cramps, and in extreme cases can be known to be extremely and unimaginably painful.[8] Additionally, some women may experience bloating, lower back pain, food cravings, fatigue, and headaches shortly before menstruation or while menstruating.[9]

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[6] Melissa K. Ochoa, "The Question You Should Never Ask Women — Period," The Conversation, June 4, 2019.
[7] "Premenstrual Syndrome (PMS) — Symptoms & Causes," Mayo Clinic, Feb. 25, 2022.
[8] Lori Smith, "All You Need to Know about Period Symptoms," Medical News Today, updated June 21, 2023.
[9] National Institute of Child Health and Human Development, "What Are the Symptoms of Menstruation?," NICHD, last reviewed Jan. 31, 2017.

2.2.2 Physical Effects in Education

Under the educational scene, the physical symptoms of menstruation may be particularly prominent when students are in school. Severe abdominal cramping may affect a student's academic performance during examinations or tests, or constant fatigue can also hinder their ability to pay attention in class.[10] There are a multitude of ways that these physical symptoms may affect a student's ability to engage in class, and not all implications are covered; however, it is important to acknowledge that physical symptoms are unavoidable and inevitably will impact academic performance.

Heating pads, for example, can help relieve abdominal cramping or pain by relaxing the muscles and offer immediate alleviation.[11] Medication, too, can help soothe pain from headaches or cramping; if district regulations allow, providing medication for students can also be a supplement implemented into nurse offices or other regulated locations that not only helps women feel more comfortable at school but also allows them to better engage in their classes.

[10] Sulagna Maity et al., "Academic and Social Impact of Menstrual Disturbances in Female Medical Students: A Systematic Review and Meta-Analysis," Frontiers in Medicine 9 (2022).
[11] Sami Sharfin, "The Benefits of Using a Heating Pad for Cramps," Aunt Flow, Apr. 7, 2024.

3. Regulatory Requirements and Curricula

This section highlights the importance of menstrual education, the current status of menstrual education and how it may be improved. Comprehensive menstrual education equips students with accurate information to make educated decisions regarding their hygiene and health and mitigating the fear and anxiety youth may feel during a transformative period of their lives.

3.1 Importance of Schools

Research done in Chicago, Los Angeles, and New York City reveals that many parents turn to schools to educate their children on puberty and sexual health because they personally feel discomfort or believe their knowledge is inadequate. This study found that many girls desired more education on menstruation with an emphasis on practical guidance, yet were hindered by teachers' lack of knowledge or discomfort and poor experiences with previously disengaging or limited menstruation and puberty education[12].

3.2 Status Quo Analysis

Health education in New York State is primarily governed by Commissioner's Regulation 135.3 (8 NYCRR § 135.3), which requires both elementary and secondary school curricula to include health education for all students[13], and mandates one-half unit of health education credit for a high school diploma under § 100.5 of the Commissioner's Regulations[14]. However, the only sexual health topic explicitly mandated by these regulations is HIV/AIDS education. The state's non-binding Learning Standards for Health, Physical Education, and Family and Consumer Sciences serve as another framework for educational institutions, yet mention "understanding of the changes that accompany puberty" as a topic area but do not name menstruation explicitly[15]. Menstrual education often receives substandard and limited attention within broader health curricula, and is sometimes treated as a one-time "special presentation". While some districts provide in-depth explanations, implementation remains uneven across the state.

3.3 Proposed Legislation

Menstrual health-related legislation has been expanded in the past years, and much of menstrual health related legislation addresses access and health coverage rather than

[12] Margaret L. Schmitt et al., "'It Always Gets Pushed Aside:' Qualitative Perspectives on Puberty and Menstruation Education in U.S.A. Schools," Frontiers in Reproductive Health 4 (2022).
[13] N.Y. Comp. Codes R. & Regs. tit. 8, § 135.3 (2018).
[14] N.Y. Comp. Codes R. & Regs. tit. 8, § 100.5.
[15] New York State Education Department, Learning Standards for Health, Physical Education, and Family and Consumer Sciences at Three Levels.

classroom content. This brief, therefore, emphasizes Senate Bill 6227 / Assembly Bill 7557 because it concerns the health curriculum specifically. In the 2026 Legislative Season, it passed the Senate but not the Assembly. This bill would authorize the Commissioner of Education to issue rules adding menstrual disorders like endometriosis and information on the importance of menstrual health to the health curriculum[16]. Endometriosis, for example, is often undiagnosed because young women assume its symptoms are normal and come with menstruation as a whole.

3.4 Alternatives and Recommendations

It is this brief's belief that specific instructions must be given by the state to ensure menstrual and reproductive health are taught in a manner that informs young students thoroughly about their own bodies and dispels the fears they may have about this process.

First, this brief urges the reintroduction and passage of S. 6227 / A. 7557 to ensure menstrual disorders and the importance of menstrual health are taught to students. Mandating certain topics be explicitly taught will help bridge the gap in implementation that exists currently as a result of vague legislation.

Second, menstruation should be explicitly added to the Learning Standards for Health with a recommendation that students have knowledge of how their physical and mental health will be affected, methods of navigating through menstruation and hormonal changes, and basic symptoms of common menstrual disorders. This would work in tandem with S. 6227, as it would provide more details and be a more thorough guide to schools that have not yet included menstrual health in their class curricula.

Finally, teacher instruction on menstrual health and gender-sensitive instruction should be mandated. Schools should provide workshops and resources for their teachers to become more comfortable discussing with students, giving out accurate information, and creating engaging lessons.

[16] S. 6227, 2025–2026 Reg. Sess. (N.Y. 2025); A. 7557, 2025–2026 Reg. Sess. (N.Y. 2025).

4. Classroom Destigmatizing Measures

Stigma surrounding menstrual health hinders the effectiveness of reproductive health education by discouraging open discussion and increasing misinformation. Though menstruation is a normal physiological process, social stigma has contributed to gaps in menstrual health literacy, especially among adolescents. Comprehensive reproductive health education can reduce stigma, increase health literacy, and bolster confidence in managing menstrual health.

4.1 Discussion Opportunities

Inadequate education and limited discussion are significant contributors to menstrual stigma and misinformation. If one doesn't understand an issue, they are much more likely to shun it when others attempt to talk about it. There does not yet exist a required mechanism for anonymous question opportunities or other instructional methods designed to promote the discussion of sensitive health topics, contributing to stigma by limiting opportunities for students to normalize discussion of reproductive health within educational settings.

4.2 Gender Separation in Health Classes

New York statute does not specify separate or co-educational discussions of puberty or menstrual content, leaving the decision to each individual school. While separated discussions are intended to increase students' comfort, they end up creating secrecy and stigma. Separate spaces, such as a teacher's office hours or an in-school community, may be created to help students who do not feel comfortable asking private questions in front of many peers, but separating health classes as a whole frames menstruation as something meant to be hidden. Research from 2026 evaluated a dual-approach school-based educational intervention, featuring co-educational lessons for foundational knowledge and tackling stigma as well as girls-only sessions to discuss symptom management and healthcare access. This program was designed to improve menstrual health knowledge and attitudes among high school students, and the conclusion revealed significant improvements in boys' perceptions of menstruation as a natural process and a decreased likelihood of girls denying experiencing effects of menstruation[17].

[17] Laura Bollinger, Géraldine Escriva-Boulley, and Nathalie Gavens, "Evaluating a School-Based Educational Intervention on Boys' and Girls' Menstrual Health Knowledge and Attitudes," Health Education Journal 85, no. 4 (2026).

4.3 Alternatives and Policy Recommendations

Menstrual education must not just aim to provide reliable sexual and reproductive health information, but should also work towards destigmatizing such topics. This brief recommends the following measures:

First, students should be provided opportunities to submit questions concerning menstruation, reproductive health, or related instructional content anonymously. Educators should discuss submitted questions during instruction while preserving student anonymity. Questions implying suspected abuse, neglect, or circumstances requiring mandatory reporting will be addressed in accordance with New York Social Services Law § 413.[18] Questions that are deemed intentionally disruptive, discriminatory, or unrelated to instructional objectives will be excluded at the educator's professional discretion.

Second, health classes should be co-educational instead of separate. These classes should not only discuss menstruation itself but also the presence and effects of social stigma towards menstruation. Students should be encouraged to discuss why this stigma exists, why it is unreasonable, and how they can create a more welcoming and open-minded community. Separate spaces may be created for girls to pursue even more detailed information or ask questions regarding their own personal experience, but the classroom curriculum should be taught to all genders, together.

[18] N.Y. Soc. Serv. Law § 413.

5. Menstrual Product Accessibility

Menstrual product accessibility is essential to students' full participation in their education, and convenient access prevents unnecessary interruptions to learning. While New York State has already made significant progress in this area in recent years, existing policy still contains gaps and oversights. This section examines existing legal frameworks for menstrual product provision in educational settings and offers recommendations to increase accessibility and sanitation standards.

5.1 Status Quo Analysis

New York Public Health Law § 267 requires that elementary and secondary public and non-public schools serving students in grades six through twelve provide menstrual products in school restrooms at no cost to students. Schools themselves determine what types of products to make available through discussion with a school nurse or other health care professional, and products are accessible in a way that ensures student privacy, including within gender-neutral bathrooms.[19] This law was amended on July 1, 2024, updating terminology from "feminine hygiene products" to "menstrual products" and extending the requirement to non-public schools.[20]

Despite this legislative progress, limitations still remain: this law currently only applies to grades six through twelve, leaving out students in elementary schools that may begin menstruating at a younger age. Though the New York City Department of Education has expanded availability to students in grades four and five, this expansion is not mandated statewide.[21] In addition, this law does not establish standards for product quality or variety. There are no requirements for schools to offer multiple options, to maintain sanitary dispensers, or restock them at certain intervals. There are also little to no inspections of school restrooms to ensure compliance, leaving actual implementation varying from well-stocked dispensers refilled monthly to a labeled but empty box on the wall.

5.2 Quality of Supplies

The quality of menstrual hygiene supplies provided in schools is vital to student health. Empirical research suggests lower quality menstrual products have negative consequences: it resulted in women using products for over-extended periods of time, increasing their risk of Toxic-Shock-Syndrome (TSS), decreased accessibility for women who had disabilities, and overall

[19] New York State Education Department, *Non-Public Schools Menstrual Products Exemption Form*.
[20] N.Y. Public Health Law § 267.
[21] N.Y.C. Pub. Schs., Administration, INFOHUB.

gave women anxiety, afraid that their period would leak through their clothes.[22] Therefore, it is important to ensure that schools are able to provide safe and high-quality products for students who menstruate.

Recent findings have proven that specific types of menstrual hygiene supplies, such as tampons, contain potentially toxic and harmful chemicals to the human body. Specific brands of tampons can contain toxic properties like lead, arsenic, and cadmium: these metals were found to increase the risk of dementia, infertility, diabetes, cancer, hinder fetal development, and many other health risks.[23] Other hygiene products mirror these risks: pads, too, host harmful chemicals to the human body and can lead to infections when not used properly.[24] Therefore, it is imperative that schools are aware of the harm specific brands/types of menstrual hygiene products pose, and develop efforts to combat these issues.

5.3 Alternatives and Policy Recommendations

This brief recommends the following measures to strengthen menstrual product accessibility in New York schools:

First, grade-level provision should be expanded. New York Public Health Law § 267 should be amended to require menstrual products in all elementary and secondary schools serving students in grades four through twelve, as it would ensure that students who begin menstruating before sixth grade have access to the necessary products within their schools, preventing disruptions to students' education and reducing stigma associated with early menstruation.

Second, product variety should be mandated. Schools should be required to provide a minimum of two types of menstrual products to accommodate individual student needs and preferences. Product selection should be made in consultation with a school nurse or health care professional, with consideration for students with sensitivities or specific health needs.

Finally, sanitary standards for product storage and restroom facilities should be established. The New York State Education Department, in consultation with the Department of Health, should issue regulations requiring regular inspection of product dispensers and private restroom environments with appropriate disposal bins.

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[22] Margaret L. Schmitt et al., "'It Always Gets Pushed Aside:' Qualitative Perspectives on Puberty and Menstruation Education in U.S.A. Schools," Frontiers in Reproductive Health 4 (2022).
[23] Elise Proulx, "First Study to Measure Toxic Metals in Tampons Shows Arsenic and Lead, among Other Contaminants," Berkeley Public Health (2024).
[24] Campaign for Safe Cosmetics, "Menstrual Care Products & Toxic Chemicals," *Safe Cosmetics*, n.d.

6. Conclusion

The 2027 Legislative Season of the New York State Legislature has the power to create policy changes that promote education about menstruation and women's issues to equip our people with the knowledge to take full control of and advocate for their personal health and stamp out stigma before it has time to take shape. We urge policymakers to listen, learn, and act upon the issues addressed in this policy brief and the issues that we, as female students, face daily at school. We hope menstruation is no longer treated as a secret to hide, but a natural adolescent experience void of fear or shame. We hope to foster empathy and understanding among youth and break the silence surrounding these issues. We hope that this policy brief has helped educate others about the importance of discourse surrounding menstruation and that the impact of this brief materializes in the next legislative season.